



GAGAN PATHOLOGY & IMAGING PVT. L.

(Previously Gagan Pathology Centre)

MC-6091

NABH (MIS) Accredited CT / MRI Imaging

MRI, CT, ULTRASOUND, ECHO, TMT, EEG, EMG
MAMMOGRAPHY, NCV, DEXA SCAN, OPG, ECG
PFT, BERA, VEP, DIGITAL & PORTABLE X-RAYS

Lab Report

Patient Name	Mrs. RANJANA	Patient Id	1026110650
Sex / Age	Female /23 Yrs	Registered On	27/08/2026 15:58:16
Ref. Dr.	Dr. SELF*	Receiving On	27/08/2026 16:07:17
Sample From	SUMAN WELLNESS*ASHOK VIHAR	Printed On	27/08/2026 20:44:04
Sample	SERUM	Barcode	

Test Name	Value	Unit	Biological Ref Interval
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TSH ultrasensitive	2.50	μIU/ml	0.35 - 5.5
Method : CLIA			

Above mentioned reference ranges are standard reference ranges for adults

AGE RELATED GUIDELINES FOR REFERENCE RANGES FOR TSH

TSH	NEW BORN	INFANT	CHILD
(u lu/ml)	1.0-38.9	1.7-9.1	0.7-6.4

Note: TSH levels are subject to circadian variation, reaching peak levels between 2 - 4 AM and at a minimum between 6 - 10 PM. The variation is of the order of 50%, hence time of the day has influence on the measured serum TSH concentrations. Dose and time of drug intake also influence the test result.

IIIrd Generation Ultrasensitive kits are used.

Serum FT3, FT4 and TSH measurements form three components of thyroid screening panel and are useful in diagnosing various disorders of thyroid gland function.

1. Primary hyperthyroidism is accompanied by elevated serum T3 & T4 values alongwith depressed TSH level.
2. Primary hypothyroidism is accompanied by depressed serum T3 and T4 values & elevated serum TSH levels.
3. Normal T4 levels accompanied by high T3 levels and low TSH are seen in patients with T3 thyrotoxicosis.
4. Normal or low T3 & high T4 levels indicate T4 thyrotoxicosis (problem is conversion of T4 to T3)
5. Normal T3 & T4 along with low TSH indicate mild / subclinical HYPERTHYROIDISM .
6. Normal T3 & low T4 along with high TSH is seen in HYPOTHYROIDISM .
7. Normal T3 & T4 levels with high TSH indicate Mild / Subclinical HYPOTHYROIDISM .
8. Slightly elevated T3 levels may be found in pregnancy and in estrogen therapy while depressed levels may be encountered in severe illness , malnutrition , renal failure and during therapy with drugs like propranolol.
9. Although elevated TSH levels are nearly always indicative of primary hypothyroidism . rarely they can result from TSH secreting pituitary tumours (secondary hyperthyroidism).

Result is to be correlated clinically

Dr. R.K. Garg

MBBS, MD
Senior Consultant Pathologist

FA FINAUT

Dr. Ankur Garg

MBBS, MD
Consultant Pathologist

Expert and Reliable Opinion for over 30 Years

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On Panel : CGHS, D.G.E.H.S., DJB, NDPL, ECHS, DDA, ITPO, NDMC, MCD, LIC (TPA), ESI, DELHI UNIVERSITY



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*** End of Report ***

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